

Mid-Urethral Sling (for Stress Incontinence)

Patient Information · A common surgery for stress urine leakage

WARWIKI

A mid-urethral sling is the most common surgery for **stress urinary incontinence** in women, which is leaking when you cough, laugh, lift or exercise. A narrow strip of mesh is placed under the middle of the urethra to support it. It is usually done as a same-day surgery.

About This Procedure

Through a small cut inside the vagina, a thin tape is placed loosely under the urethra, like a hammock. When you cough or strain, the urethra presses against the tape and stays closed. There is **nothing to operate** afterward.

The tape can take different paths: behind the pubic bone (retropubic), out through the groin (transobturator), or a short "mini" sling. Your surgeon will talk with you about which is best.

Benefits and Risks in Numbers

about
8 in 10

have no leakage on testing 1 year later (fewer feel fully dry)

1–5 in 100

have a bit of mesh show through the vaginal wall, which can be treated

about
6 in 100

get groin or thigh pain after a groin-route sling, usually short-lived

Numbers are averages from published studies. Your own risk may differ; ask your team.

About the Mesh

The mesh sling is one of the most studied surgeries for women. In 2019 the FDA stopped the sale of vaginal mesh for **prolapse**. That order did not apply to slings for leakage. A sling is still a permanent implant, with risks of mesh showing through, pain and urinary problems. If you prefer no mesh, a sling made from **your own tissue** or a Burch repair are other choices.

Learn the Terms

Stress incontinence

Leaking urine with pressure, such as coughing, laughing or lifting.

Urethra

The tube that carries urine out of the body.

Mid-urethral sling

A mesh tape that supports the middle of the urethra.

Retropubic sling

A sling that passes behind the pubic bone.

Transobturator sling

A sling that passes out through the groin.

Autologous sling

A sling made from your own tissue instead of mesh.

Mesh exposure

When a bit of mesh shows through the vaginal wall.

Cystoscopy

A look inside the bladder with a thin camera.

Will it hurt? You will be asleep or numb during surgery, so you feel nothing. Afterward, expect mild soreness and a little spotting. Most people go **home the same day**.

Before you leave

You urinate to show your bladder empties. If not, a catheter goes home briefly.

First 2 weeks

Light activity is often advised. Walking is fine.

After that

Return to your usual activities as you feel able.

Follow-up visit

Your team checks your leakage and emptying.

How to Get Ready

- Follow your surgery instructions about eating, drinking and which medicines to stop, including blood thinners.
- A **urine test** rules out infection. Some people have bladder testing first.
- Arrange a ride home.

What Happens

- 1 You are asleep or numb, and you get antibiotics.
- 2 The tape is placed loosely under the middle of the urethra through a small vaginal cut.
- 3 The surgeon looks inside your bladder with a camera to check it.
- 4 The small skin cuts above the pubic bone or in the groin are closed.

After Surgery

- Long, strict limits on activity have not been shown to help the sling work better. Your team will tell you what to avoid, and for how long.
- Ask your team when you can have sex or use tampons.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- A weak stream, or a feeling your bladder is not emptying
- Pain that gets worse, or later pain with sex or new vaginal discharge

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- You have heavy bleeding that soaks a pad an hour
- You cannot reach the office and are worried

Three Things to Remember

1. A mid-urethral sling supports the urethra to stop stress leakage. There is nothing to operate.
2. It is a permanent mesh implant. The FDA prolapse mesh order does not apply to it.
3. You can return to usual activity fairly soon. Follow your team's advice.